

ClinicalIQ AI-Assisted Clinical Report

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Uncertainty Flags: None detected

Chief Complaint

Symptoms: polyuria,polydipsia,fatigue,weight loss

Clinical Summary: 2 weeks of polyuria, thirst, fatigue, and weight loss

Vitals

Vital	Value	Range	Status
Heart Rate	116 bpm	60-100 bpm	Abnormal
SpO2	94 %	95-100 %	Abnormal
Temperature	38.2 C	36.1-37.2 C	Abnormal
Systolic BP	146 mmHg	90-120 mmHg	Abnormal

Disease Ranking



Recommended Tests by Priority

Priority 1: Fasting Plasma Glucose, HbA1c

Priority 2: TSH, Free T4, Urine Albumin-Creatinine Ratio, 12-Lead ECG, Lipid Panel, High-Sensitivity Troponin

Priority 3: Anti-TPO Antibody, C-Peptide, BNP/NT-proBNP, Thyroid Ultrasound, Echocardiogram, Radioiodine Uptake Scan, Cardiac Stress Test

Cost Breakdown

Test	Priority	Net Cost (USD)
Fasting Plasma Glucose	1	25.98
HbA1c	1	38.98
TSH	2	38.98
Free T4	2	45.47
Urine Albumin-Creatinine Ratio	2	49.37
12-Lead ECG	2	58.46
Lipid Panel	2	58.46
High-Sensitivity Troponin	2	103.94

Anti-TPO Antibody	3	84.45
C-Peptide	3	97.44
BNP/NT-proBNP	3	123.42
Thyroid Ultrasound	3	220.86
Echocardiogram	3	415.74
Radioiodine Uptake Scan	3	493.70
Cardiac Stress Test	3	545.66
Total		2400.91

Burden Gauge



Burden Score: 99.44/100 (High)

Top PubMed References

1. Diabetes Mellitus Symptoms and HbA1c Correlation (PMID: PMID-1001, Score: 0.00)
2. Thyroid Dysfunction and Metabolic Fatigue Patterns (PMID: PMID-1004, Score: 0.00)
3. Cardiac Ischemia Early Warning Signals (PMID: PMID-1002, Score: 0.00)

Doctor Override Summary

Confirmed Disease: diabetes

Confirmed Tests: HbA1c, Fasting Plasma Glucose, Lipid Panel

Notes: override for staging

Accuracy Tracking

Composite Score: 0.8000

Top-1 Match: 1.00

Top-3 Match: 1.00

Test Overlap: 0.20

Next Steps Timeline

0-24h: Validate high-priority tests and acute red flags.

24-72h: Review pending diagnostics and adjust test bundle if needed.

3-7 days: Reassess burden, disease ranking movement, and treatment planning.

Disclaimer

This report is AI-assisted clinical decision support. All disease rankings, test recommendations, and burden scores are advisory. Final clinical decisions rest with the attending physician.